## 1.3. Schedule of Activities (SoA)

Alternative study measures during the COVID-19 Pandemic and for Extenuating Circumstances is described in Appendix 13.

| **Procedures Year 1** | **Screening Period** | **Baseline Period** | **IP Infusion** | **Week 1-12 (± 2 days)** | | | | | **Week 13-52 (±1 week)** | | | | | • **The following Screening labs should be collected in the absence of residual infused FIX (FIX trough): FIX activity, hemostasis markers, inhibitor testing.**<br>• The Screening and Baseline period may be extended, upon consultation with the sponsor's medical monitor. Reason must be recorded in the source documents. Depending on the length of extension (of either period), the medical monitor may request to have visit procedures repeated.<br>• After determination of eligibility, FIX replacement therapy and IP Infusion will be planned at a time when a routine FIX infusion will occur during the study visit on Day 1.<br>• Study sites without necessary infusion facilities will complete Visits 1-2 at the study site and Visit 3 (Day 1) will be completed at the designated Infusion Center.<br>• Day 1: All eligible participants will be dosed on Day 1 and are required to return to the infusion center or study site 24 hours after being dosed (on Day 2) to have their vital signs assessed.<br>• ∗Visit 4: The participant is not required to appear in person at this visit. Visit 4 study procedures are not required to be performed if the participant does not appear in person with the exception of all required laboratory specimens (eg, Immunology – Exploratory [IgG vs IgM antibody response], Vector Shedding Analysis by PCR, FIX Activity, Antigen and LFT) which can be obtained via mobile phlebotomy home health services. If participants physically attend this visit, all procedures are to be performed per SOA.<br>• Unscheduled visits may be necessary during the study for safety monitoring or to repeat any blood sampling if required.<br>• Mobile phlebotomy/home health services may be utilized for Pre-Dose LFT blood draws and most blood collections after Study Day 1/Visit 3 infusion, as needed. Additionally, mobile phlebotomy/home health service may be utilized, prior to Visit 3, upon sponsor approval, except for pre-dose LFT blood draws which do not require sponsor approval.<br>• Early discontinuation visit should follow the procedures for Week 312 visit. The management of the participant and the risks inherent with discontinuation from the study should be discussed with the sponsor's medical monitor. |
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| **Study Visit** | 1 | 2 | 3 | 4* | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 | |
| **Weeks after IP administration**<br><br>The schedule of assessments should be based on Day 1 | Week -6 to Week -3 | Week -3 to Day 1 | Day 1 | 1 | 2 | 4 | 8 | 12 | 18 | 24 | 32 | 42 | 52 | |
| Informed Consent | X | | | | | | | | | | | | | Must be obtained from participants prior to any study-related procedures. Two ICFs may be required, one from the Study Site and the other from the Infusion Center (if applicable). |
| Assess/ confirm eligibility | X* | | X# | | | | | | | | | | | * Review and confirm eligibility criteria (see Sections 5.1 & 5.2). This needs to be completed prior to ordering IP at the beginning of the Baseline Period.<br># Review and confirm eligibility (see Sections 5.1 & 5.2) prior to infusion (Infusion Center should confirm eligibility with study site). |
| Demographics, Medical, Surgical, and Hemophilia History (including vaccination history) | X | | | | | | | | | | | | | All ongoing AEs and SAEs in the lead-in study (C0371004) including events of special interest, as defined in the lead-in study (C0371004), will be collected as medical history for this study. Only changes to historical data on Demographics, Medical, Surgical and Hemophilia History will be captured. Any new AEs/SAEs after completion of the lead-in study will follow the AE reporting process (Section 8.3.1, Appendix 3). |

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| **Weeks after IP administration**<br><br>The schedule of assessments should be based on Day 1 | Week -6 to Week -3 | Week -3 to Day 1 | Day 1 | 1 | 2 | 4 | 8 | 12 | 18 | 24 | 32 | 42 | 52 | |
| Physical Exam, Weight, and Vital Signs | X | X | X | X | X | X | X | X | X | X | X | X | X | Complete Physical Exam at all clinic visits (Height is required to be collected at Screening Visit). Weight must be obtained at Screening Visit to order the correct quantity of IP. Weight at Screening Visit (Visit 1) will be used to calculate the dose (Section 8.2.1).<br>**Vital signs include**: body temperature (°C) (see Section 8.2.2), BP, pulse rate, respiratory rate, after at least 5 mins rest in supine or upright/sitting position.<br>Vital signs are to be measured as specified below.<br>**Vital signs (at Day 1)** will be obtained:<br>• prior to **FIX infusion**;<br>• 30±2 mins **after the start of IP infusion (taken during infusion)**;<br>• approximately 2 mins, 2 hrs ±10 mins, 6 hrs ±10 mins, and 24 (±3) hrs **after completion of IP infusion and IV line flush**. |
| Electrocardiogram | X | | | | | | | | | | | | X | 12-lead ECG will be obtained for the study. Additional ECGs may be obtained as clinically indicated. |
| Target Joint Assessment | X | | | | | | | | | | | | | See Appendix 7 for definition of a target joint.<br>Done as part of hemophilia history. |

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| **Study Visit** | 1 | 2 | 3 | 4* | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 | |
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| Joint X-ray | | X*# | | | | | | | | | | | | *Some participants (n~40), who consent to participate in an optional sub-study, will undergo X-ray for assessment of joints (Section 8.1.2.3). For German sites please refer to Germany Appendix (Section 10.11.2.4).<br># If this X-ray can't be performed during the baseline period, it is acceptable to postpone this testing no later than Week 4/ Visit 6. |
| Joint MRI | | X*# | | | | | | | | | | | | *Some participants (n~20), who consent to participate in an optional sub-study, will undergo joint MRIs (Section 8.1.2.2).<br># If this MRI can't be performed during the baseline period, it is acceptable to postpone this testing no later than Week 4/ Visit 6. |
| HBcAB, HCV, CD4, human immunodeficiency virus (HIV) Serology, Liver fibrosis testing & Urinalysis (CL*) | X | | | | | | | | | | | | | Hepatitis: HBsAg, HBcAb, HBV-deoxyribonucleic acid (DNA); HCV-RNA Quantitative<br>Liver fibrosis: FibroScan, FibroTest/FibroSURE, or AST to platelet ratio index (APRI).<br>* FibroScan, if performed, would be performed locally.<br>HIV: Antibody testing, and CD4 count/HIV viral load (if antibody positive).<br>Urinalysis: pH, specific gravity, protein, blood, ketones, glucose. |
| α-Fetoprotein (CL) | X | | | | | | | | | | | | X | To be tested as biomarker for hepatic carcinoma. |
| FIX genetic testing (CL) | X* | | | | | | | | | | | | | *Test to be performed if results are not available in the source documents. |

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| FIX Inhibitor testing (CL, test in LL if clinically necessary) | X* | | | | X | X | X | X | X | X | X | X | X | **These specimens should be collected in the absence of residual infused FIX.**<br>*Positive inhibitor testing as measured by the central laboratory ≥0.6 Bethesda Units (BU) at Visit 1 would be exclusionary.<br>Nijmegen Bethesda Inhibitor assay at the central lab unless testing at a local lab is necessary due to a clinical concern. |
| nAb Assay to adeno-associated virus (AAV)-Spark 100 (CL) | X* | | | | | | | | | | | | X | *Participants with Anti-AAV-Spark100 neutralizing antibodies (nAb) titer ≥ 1:1 (ie, positive for nAb) are to be excluded from the study. |
| Anti-Drug Antibody/ADA (CL) | X | | | | | | | | | | | | X | Anti-PF-06838435 antibodies (ADA) from human serum. |
| Laboratory Safety Panels (Hematology and Clinical Chemistry) (CL) | X | | | | X | X | X | X | X | X | X | X | X | •Hematology: WBC and differential, RBC, hemoglobin, hematocrit, and platelet count.<br>•ABO blood group (CL) (at Screening if unknown).<br>•Chemistry: Na, K, Cl, phosphate, bicarbonate, glucose, BUN, serum creatinine. |
| Laboratory Safety Panels (Lipid Panel) (CL) | X | | | | | | | | | | | | X | These specimens are to be drawn after at least 8 hours of fasting (Appendix 2). |

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| FIX Activity, LFT (CL, LL) and<br>FIX Antigen (CL)* | X | | | Collect 2-3 times every week starting from Day 1. Utilize mobile phlebotomy/ home health services as needed. | | | | | <u>Week 13 to Week 18</u>: Collect once every week. Utilize mobile phlebotomy/ home health services as needed.<br><u>Week 19 to Week 52</u>: Collect at site visits. | | | | | • LFT: albumin, total bilirubin, direct bilirubin, indirect bilirubin (if available), ALP, AST, ALT, total protein, GGT, LDH.<br>• FIX:C, FIX Ag.<br>• **FIX Activity should be collected in the absence of residual infused FIX.**<br>• For first 12 weeks, 2 or 3 times every week monitoring will be conducted due to potential rise in hepatic transaminases and/or loss of FIX transgene expression.<br>• Sponsor recommends collecting specimens on Mondays through Thursdays so that the results are available prior to the weekend.<br>• *** FIX Activity and LFT testing will be conducted at local AND central lab for all time points and as clinically necessary (including samples drawn at the clinic and any mobile phlebotomy/ home health service visits)**. FIX Antigen testing will be conducted ONLY at central lab for all time points and as clinically necessary. Clinical decisions may be based on local and/ or central lab results. |
| Order the IP | | X | | | | | | | | | | | | IP is to be ordered at the beginning of the Baseline Period after confirming eligibility criteria. The IP will take approximately 3 weeks for delivery. Prior to ordering IP, Sections 10.13.3 and 10.13.4 should be considered. |

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| Immunology (PBMC) (CL) | | X | | Only if corticosteroid treatment is needed for a suspected T-cell response.<br>Utilize mobile phlebotomy/ home health services as needed and if possible. | | | | | | | | | | •Specimens for PBMC by ELISPOT are to be collected at Baseline and if the corticosteroid treatment is needed for a suspected T-cell response.<br>•Specimens are to be collected prior to initiating the corticosteroid treatment. If not collected prior to initiation, specimens must be collected within 24 hours of administering corticosteroids.<br>•It is highly recommended that participants who are initiated on corticosteroid treatment be treated with a gastric acid reducer, preferably a proton pump inhibitor (PPI) (eg, omeprazole), or alternatively a histamine type 2 (H2) antagonist (eg, ranitidine) for the duration of the corticosteroid course.<br>•Specimens are to be collected approximately 3 weeks after steroid administration. Additional specimens may be necessary if ELISPOT has not normalized at 3 weeks and/or based on medical judgement. |
| Immunology Exploratory (PBMC) (CL) | X | | | | X | X | X | X | | | | | X | Utilize mobile phlebotomy/ home health services, as needed and if possible.<br>Collect blood for PBMC preparation for cell-mediated exploratory immunology assays. |

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| Immunology - Exploratory (binding IgG vs IgM antibody response) (CL) | | X | | X* | X | | | | | | | | | Serum samples will be used for these tests.<br>Post-dose samples will be collected on Week 1 and Week 2.<br>*Specimen is required at Visit 4 and can be obtained via mobile phlebotomy/ home health service. |
| Spare Plasma | X | X | X | X | X | X | X | X | X | X | X | X | X | Approximately 5 mL of blood will be drawn to provide approximately 2 mL of plasma, each time blood is collected for central laboratory samples. The plasma will be stored for repeat or additional testing. These samples will be destroyed at the end of the study. |
| Liver Ultrasound | X | | | | | | | | | | | | X | All participants will undergo liver ultrasound. Imaging will be performed locally (Section 8.2.5). |
| Global Hemostasis Markers (CL) | X | | | | | | | | | | | | X | aPTT (in seconds), INR, TAT, TGA, and D-dimer,<br>**These specimens should be collected in the absence of residual infused FIX**.<br>The tests may be conducted as clinically indicated throughout the study. |
| Issue the eDiary | X | | | | | | | | | | | | | Provide eDiary and training to operate it, including timing and information to be entered starting at Visit 1 (see Section 8.1.1). |
| Review the eDiary | | Ongoing (including unscheduled visits)# | | | | | | | | | | | | Review the eDiary data (see Section 8.1.1), ensure appropriate entries, during unscheduled and scheduled visits.<br># Infusion centers are not required to perform this activity on Visit 3; however, the study site is required to perform this review. |

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| Pre-Dosing LFTs | | | X | | | | | | | | | | | Blood for LFTs are to be drawn and tested locally and centrally prior to dosing participants (up to 3 days prior to Day 1/ Visit 3 is acceptable). Utilize mobile phlebotomy/ home health services as needed, and if possible.<br>Local LFTs are required to be reviewed prior to thawing drug. If the investigator has any clinical concerns regarding the local LFT results, drug should not be thawed. These LFT results may help in the assessment of future LFTs and decisions on when to begin corticosteroid treatment, if necessary. These LFT results should be entered into the CRF as an unplanned visit. |
| FIX Infusion, Accountability | | | X | | | | | | | | | | | On Day 1 a single prophylactic IV infusion of a FIX product will be administered under medical supervision, over a period of approximately 10 minutes at a dose of 100 IU/kg for standard half-life products and 50 IU/kg for extended half-life products**, within reasonable margin of error (approximately ±20%)**. If there is evidence to show that these doses may place the participant at supratherapeutic FIX levels, the dose can be adjusted.<br>Participants are expected to bring their current Factor IX replacement therapy with them. |

| **Procedures Year 1** | **Screening Period** | **Baseline Period** | **IP Infusion** | **Week 1-12 (± 2 days)** | | | | | **Week 13-52 (±1 week)** | | | | | |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| **Study Visit** | 1 | 2 | 3 | 4* | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 | • **The following Screening labs should be collected in the absence of residual infused FIX (FIX trough): FIX activity, hemostasis markers, inhibitor testing.**<br>• The Screening and Baseline period may be extended, upon consultation with the sponsor's medical monitor. Reason must be recorded in the source documents. Depending on the length of extension (of either period), the medical monitor may request to have visit procedures repeated.<br>• After determination of eligibility, FIX replacement therapy and IP Infusion will be planned at a time when a routine FIX infusion will occur during the study visit on Day 1.<br>• Study sites without necessary infusion facilities will complete Visits 1-2 at the study site and Visit 3 (Day 1) will be completed at the designated Infusion Center.<br>• Day 1: All eligible participants will be dosed on Day 1 and are required to return to the infusion center or study site 24 hours after being dosed (on Day 2) to have their vital signs assessed.<br>• *Visit 4: The participant is not required to appear in person at this visit. Visit 4 study procedures are not required to be performed if the participant does not appear in person with the exception of all required laboratory specimens (eg, Immunology – Exploratory [IgG vs IgM antibody response], Vector Shedding Analysis by PCR, FIX Activity, Antigen and LFT) which can be obtained via mobile phlebotomy home health services. If participants physically attend this visit, all procedures are to be performed per SOA.<br>• Unscheduled visits may be necessary during the study for safety monitoring or to repeat any blood sampling if required.<br>• Mobile phlebotomy/home health services may be utilized for Pre-Dose LFT blood draws and most blood collections after Study Day 1/Visit 3 infusion, as needed. Additionally, mobile phlebotomy/home health service may be utilized, prior to Visit 3, upon sponsor approval, except for pre-dose LFT blood draws which do not require sponsor approval.<br>• Early discontinuation visit should follow the procedures for Week 312 visit. The management of the participant and the risks inherent with discontinuation from the study should be discussed with the sponsor's medical monitor. |
| **Weeks after IP administration**<br><br>The schedule of assessments should be based on Day 1 | Week -6 to Week -3 | Week -3 to Day 1 | Day 1 | 1 | 2 | 4 | 8 | 12 | 18 | 24 | 32 | 42 | 52 | |
| IP Infusion, Accountability | | | X | | | | | | | | | | | Prior to dosing, Section 10.13.3 and 10.13.4 should be considered.<br>After the completion of FIX product infuion, participants will receive a single IV infusion of the IP over approximately 60 minutes via infusion pump. IP should NOT be thawed without: confirmation of eligibility, Pre-Dosing LFT results, AND affirming that the participant is physically present at the study site (or infusion center).<br>**The IV line should be flushed at the conclusion of the IP infusion.** |
| HJHS | | X | | | | | | | | X | | | X | To be completed by investigator or the designee (Section 8.1.2.1). |
| Patient-Reported Outcomes (PROs) | | X* | | | | | | X | | X | | | X | Participants will complete 5 instruments at the site using an electronic tablet: Haem-A-QoL, Haemophilia Activities List [HAL (v2)], PGIC-H, HLIQ, and EQ-5D-5L. The questionnaires should be administered before dosing, treatment, or conversation between health care team and participants about their health condition. See Section 8.1.3 for additional considerations.<br>* PGIC-H is not completed at baseline visit.<br>Every effort should be made to conduct the PROs according to the protocol specified timepoints, especially the baseline visit. Baseline PRO assessments need to be done prior to start of infusion on Day 1 if not done at baseline visit. If the post-infusion PROs can't be performed at a particular visit, they should be performed at the next visit. |

| **Procedures Year 1** | **Screening Period** | **Baseline Period** | **IP Infusion** | **Week 1-12 (± 2 days)** | | | | | **Week 13-52 (±1 week)** | | | | | |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| **Study Visit** | 1 | 2 | 3 | 4* | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 | • **The following Screening labs should be collected in the absence of residual infused FIX (FIX trough): FIX activity, hemostasis markers, inhibitor testing.**<br>• The Screening and Baseline period may be extended, upon consultation with the sponsor's medical monitor. Reason must be recorded in the source documents. Depending on the length of extension (of either period), the medical monitor may request to have visit procedures repeated.<br>• After determination of eligibility, FIX replacement therapy and IP Infusion will be planned at a time when a routine FIX infusion will occur during the study visit on Day 1.<br>• Study sites without necessary infusion facilities will complete Visits 1-2 at the study site and Visit 3 (Day 1) will be completed at the designated Infusion Center.<br>• Day 1: All eligible participants will be dosed on Day 1 and are required to return to the infusion center or study site 24 hours after being dosed (on Day 2) to have their vital signs assessed.<br>• *Visit 4: The participant is not required to appear in person at this visit. Visit 4 study procedures are not required to be performed if the participant does not appear in person with the exception of all required laboratory specimens (eg, Immunology – Exploratory [IgG vs IgM antibody response], Vector Shedding Analysis by PCR, FIX Activity, Antigen and LFT) which can be obtained via mobile phlebotomy home health services. If participants physically attend this visit, all procedures are to be performed per SOA.<br>• Unscheduled visits may be necessary during the study for safety monitoring or to repeat any blood sampling if required.<br>• Mobile phlebotomy/home health services may be utilized for Pre-Dose LFT blood draws and most blood collections after Study Day 1/Visit 3 infusion, as needed. Additionally, mobile phlebotomy/home health service may be utilized, prior to Visit 3, upon sponsor approval, except for pre-dose LFT blood draws which do not require sponsor approval.<br>• Early discontinuation visit should follow the procedures for Week 312 visit. The management of the participant and the risks inherent with discontinuation from the study should be discussed with the sponsor's medical monitor. |
| **Weeks after IP administration**<br><br>The schedule of assessments should be based on Day 1 | Week -6 to Week -3 | Week -3 to Day 1 | Day 1 | 1 | 2 | 4 | 8 | 12 | 18 | 24 | 32 | 42 | 52 | |
| Vector Shedding Analysis by PCR (CL) | | X | X* | Weekly, as needed#<br>Utilize mobile phlebotomy / home health services, as needed | | | | | | | | | | • # Plasma, PBMC, saliva, semen, and urine specimens will be collected at baseline and every week after IP administration until 3 consecutive specimens test negative for the given specimen type.<br>• Semen samples can be collected at home the night before a clinic or mobile phlebotomy/ home health service visit and stored in the participant's freezer until the clinic or mobile phlebotomy/ home health service visit, with the exception of semen samples collected as part of the optional vector shedding sub-study (see bullet point below)<br>• *Optional Vector Shedding Sub-study: Additional (2 hrs (±30 minutes), 24 hrs (±3 hrs), 72 hrs (±4 hrs) after completion of IP infusion and IV line flush on Day 1) samples (plasma, PBMC, saliva, semen, and urine) will be collected from some participants (n=12) who consent to participate in an optional sub-study. Utilize mobile phlebotomy/ home health services as needed. |
| Any Additional Safety Tests | Ongoing (as needed) | | | | | | | | | | | | | Additional laboratory testing may be conducted as deemed clinically necessary by the investigator to ensure safety of participants. |
| Adverse Events | Ongoing (including unscheduled visits) | | | | | | | | | | | | | During the Short-Term Monitoring Period (up to and including 52 weeks post-infusion) all SAEs and AEs (including medically important events) will be collected (Appendix 3). |

| **Procedures Year 1** | **Screening Period** | **Baseline Period** | **IP Infusion** | **Week 1-12 (± 2 days)** | | | | | **Week 13-52 (±1 week)** | | | | | • **The following Screening labs should be collected in the absence of residual infused FIX (FIX trough): FIX activity, hemostasis markers, inhibitor testing.**<br>• The Screening and Baseline period may be extended, upon consultation with the sponsor's medical monitor. Reason must be recorded in the source documents. Depending on the length of extension (of either period), the medical monitor may request to have visit procedures repeated.<br>• After determination of eligibility, FIX replacement therapy and IP Infusion will be planned at a time when a routine FIX infusion will occur during the study visit on Day 1.<br>• Study sites without necessary infusion facilities will complete Visits 1-2 at the study site and Visit 3 (Day 1) will be completed at the designated Infusion Center.<br>• Day 1: All eligible participants will be dosed on Day 1 and are required to return to the infusion center or study site 24 hours after being dosed (on Day 2) to have their vital signs assessed.<br>• *Visit 4: The participant is not required to appear in person at this visit. Visit 4 study procedures are not required to be performed if the participant does not appear in person with the exception of all required laboratory specimens (eg, Immunology – Exploratory [IgG vs IgM antibody response], Vector Shedding Analysis by PCR, FIX Activity, Antigen and LFT) which can be obtained via mobile phlebotomy home health services. If participants physically attend this visit, all procedures are to be performed per SOA.<br>• Unscheduled visits may be necessary during the study for safety monitoring or to repeat any blood sampling if required.<br>• Mobile phlebotomy/home health services may be utilized for Pre-Dose LFT blood draws and most blood collections after Study Day 1/Visit 3 infusion, as needed. Additionally, mobile phlebotomy/home health service may be utilized, prior to Visit 3, upon sponsor approval, except for pre-dose LFT blood draws which do not require sponsor approval.<br>• Early discontinuation visit should follow the procedures for Week 312 visit. The management of the participant and the risks inherent with discontinuation from the study should be discussed with the sponsor's medical monitor. |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| **Study Visit** | 1 | 2 | 3 | 4* | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 | |
| **Weeks after IP administration**<br><br>The schedule of assessments should be based on Day 1 | Week -6 to Week -3 | Week -3 to Day 1 | Day 1 | 1 | 2 | 4 | 8 | 12 | 18 | 24 | 32 | 42 | 52 | |
| Concomitant Medications, Surgeries, and Procedures | Ongoing (including unscheduled visits) | | | | | | | | | | | | | All concomitant therapy 30 days prior to Screening, during Screening and Short-Term Monitoring Period will be recorded up to and including 52 weeks post-infusion as per Section 6.5.<br>All procedures and surgeries (including elective surgeries) during the Screening and Short-Term Monitoring Period are to be recorded. |

Note: Study Site: Will screen and follow study participants post-infusion of PF-06838435 for the duration of the study. Some study sites may be approved to carry out infusions within their institution.

Infusion Center: A site, external from the study site, approved for the administration of PF-06838435.

AAV: adeno-associated virus; ADA: Anti-drug antibody; AE: adverse event; ALP: alkaline phosphatase; ALT: alanine transaminase; APRI: AST to Platelet Ratio Index; aPTT: activated partial thromboplastin time, AST: aspartate transaminase; BUN: blood urea nitrogen; CBC: complete blood count; CD4: cluster of differentiation or classification determinant; CL: central laboratory; CPT: cell preparation tube; Cr: creatinine; DNA: deoxyribonucleic acid; ECG: electrocardiogram; ELISPOT: Enzyme-Linked ImmunoSpot assay; EOS: End of Study; EQ-5D-5L: EuroQol, 5 dimensions, 5 levels; FIX: coagulation factor IX; FIX Ag: factor IX antigen; FIX:C: circulating FIX; HBcAb: Hepatitis B core antibody; GGT: gamma-glutamyl transferase; Haem-A-QoL: Haemophilia Quality of Life Questionnaire for Adults; HAL: Haemophilia Activities List; HBcAb: hepatitis B core antibody; HBsAg: hepatitis B surface antigen; HBV: hepatitis B virus; HCV: hepatitis C virus; HIV: human immunodeficiency virus; HJHS: hemophilia joint health score; HLIQ: Hemophilia Life Impacts Questionnaire; ICF: Informed Consent Form; IgG: immunoglobulin G; IgM: immunoglobulin M; INR: international normalized ratio; IP: investigational product; IU: international units; K: potassium; LDH: lactic acid dehydrogenase; LFT: liver function tests; LL: local laboratory; mins: minutes; MRI: magnetic resonance imaging; n: number; Na: sodium; nAb: neutralizing antibodies; PBMC: peripheral blood mononuclear cells; PCR: polymerase chain reaction; PGIC-H: Patient Global Impression of

| **Procedures Year 1** | **Screening Period** | **Baseline Period** | **IP Infusion** | **Week 1-12 (± 2 days)** | | | | | **Week 13-52 (±1 week)** | | | | |
|---|---|---|---|---|---|---|---|---|---|---|---|---|---|
| **Study Visit** | 1 | 2 | 3 | 4* | 5 | 6 | 7 | 8 | 9 | 10 | 11 | 12 | 13 |
| **Weeks after IP administration**<br><br>The schedule of assessments should be based on Day 1 | Week -6 to Week -3 | Week -3 to Day 1 | Day 1 | 1 | 2 | 4 | 8 | 12 | 18 | 24 | 32 | 42 | 52 |

- **The following Screening labs should be collected in the absence of residual infused FIX (FIX trough): FIX activity, hemostasis markers, inhibitor testing.**
- The Screening and Baseline period may be extended, upon consultation with the sponsor's medical monitor. Reason must be recorded in the source documents. Depending on the length of extension (of either period), the medical monitor may request to have visit procedures repeated.
- After determination of eligibility, FIX replacement therapy and IP Infusion will be planned at a time when a routine FIX infusion will occur during the study visit on Day 1.
- Study sites without necessary infusion facilities will complete Visits 1-2 at the study site and Visit 3 (Day 1) will be completed at the designated Infusion Center.
- Day 1: All eligible participants will be dosed on Day 1 and are required to return to the infusion center or study site 24 hours after being dosed (on Day 2) to have their vital signs assessed.
- ∗Visit 4: The participant is not required to appear in person at this visit. Visit 4 study procedures are not required to be performed if the participant does not appear in person with the exception of all required laboratory specimens (eg, Immunology – Exploratory [IgG vs IgM antibody response], Vector Shedding Analysis by PCR, FIX Activity, Antigen and LFT) which can be obtained via mobile phlebotomy home health services. If participants physically attend this visit, all procedures are to be performed per SOA.
- Unscheduled visits may be necessary during the study for safety monitoring or to repeat any blood sampling if required.
- Mobile phlebotomy/home health services may be utilized for Pre-Dose LFT blood draws and most blood collections after Study Day 1/Visit 3 infusion, as needed. Additionally, mobile phlebotomy/home health service may be utilized, prior to Visit 3, upon sponsor approval, except for pre-dose LFT blood draws which do not require sponsor approval.
- Early discontinuation visit should follow the procedures for Week 312 visit. The management of the participant and the risks inherent with discontinuation from the study should be discussed with the sponsor's medical monitor.

Change- Hemophilia; PROs: patient-reported outcomes; RNA: ribonucleic acid; SAE: serious adverse event; TAT: thrombin-antithrombin level; TGA: Thrombin generation assay; vg: vector genome; WBC: white blood cell.

Alternative study measures during the COVID-19 Pandemic and for Extenuating Circumstances is described in Appendix 13.

| **Procedures Year 2 to Year 6** | **Week 53-156 (±2 weeks)** | | | | **Week 157-260 (±3 weeks)** | | **EOS Week 312 (±3 weeks)** | • Unscheduled visits may be necessary during the study. Tests being performed are for safety monitoring purposes, for repeat safety assessments, or to repeat any blood sampling if required.<br>• During Long-Term Monitoring Period (Weeks 53 to 312), mobile phlebotomy/ home health services may be utilized as needed, when site visits are not scheduled.<br>• Early discontinuation visit should follow the procedures for Week 312 visit. The management of the participant and the risks inherent with discontinuation from the study should be discussed with the sponsor's medical monitor. |
|---|---|---|---|---|---|---|---|---|
| **Study Visit** | **14** | **15** | **16** | **17** | **18** | **19** | **20** | |
| **Weeks after IP administration** | **78** | **104** | **130** | **156** | **208** | **260** | **312** | |
| Physical Exam, Weight, and Vital Signs | X | X | X | X | X | X | X | Complete Physical Exam at all visits.<br>Vital signs include: body temperature (°C) (see Section 8.2.2), BP, pulse rate, respiratory rate, after at least 5 mins rest in supine or upright/sitting position. |
| Electrocardiogram | As needed | | | | | | | 12-lead ECG will be obtained for the study. Additional ECGs may be obtained as clinically indicated. |
| Joint X-ray | | | | X*,# | | | X*,# | *Some participants (n~40), who consent to participate in an optional sub-study, will undergo X-ray for assessment of joints (Section 8.1.2.3). For German sites please refer to Germany Appendix (Section 10.11.2.4).<br># If the baseline X-ray could not be performed ≤ Week 4/ Visit 6, the study participant does not need to do the 3 (Visit 17) and 6 year (Visit 20) follow-up X-rays. |
| Joint MRI | | | | X*,# | | | X*,# | *Some participants (n~20**)**, who consent to participate in an optional sub-study, will undergo joint MRIs (Section 8.1.2.2).<br># If the baseline MRI could not be performed ≤ Week 4/ Visit 6, the study participant does not need to do the 3 (Visit 17) and 6 (Visit 20) year follow-up MRIs. |
| α-Fetoprotein (CL) | X | X | X | X | X | X | X | To be tested as biomarker for hepatic carcinoma. |
| FIX Inhibitor Testing (CL, test in LL if clinically necessary) | X | X | X | X | X | X | X | Nijmegen Bethesda Inhibitor Assay: At central laboratory; local laboratory allowed if required for clinical reasons.<br>**These specimens should be collected in the absence of residual infused FIX.** |
| nAb Assay to AAV Spark 100 (CL) | | X | | X | X | X | X | |
| Anti-Drug Antibody/ADA (CL) | | | | X | | | X | Whole blood will be collected for determination of anti-PF-06838435 antibodies (ADA) from human serum. |
| Laboratory Safety Panels (Hematology and Clinical Chemistry) (CL) | X | X | X | X | X | X | X | • Hematology: WBC and differential, RBC, hemoglobin, hematocrit, and platelet count<br>• Chemistry: Na, K, Cl, phosphate, bicarbonate, glucose, BUN, serum creatinine. |

| **Procedures Year 2 to Year 6** | **Week 53-156 (±2 weeks)** | | | | **Week 157-260 (±3 weeks)** | | **EOS Week 312 (±3 weeks)** | • Unscheduled visits may be necessary during the study. Tests being performed are for safety monitoring purposes, for repeat safety assessments, or to repeat any blood sampling if required.<br>• During Long-Term Monitoring Period (Weeks 53 to 312), mobile phlebotomy/ home health services may be utilized as needed, when site visits are not scheduled.<br>• Early discontinuation visit should follow the procedures for Week 312 visit. The management of the participant and the risks inherent with discontinuation from the study should be discussed with the sponsor's medical monitor. |
|---|---|---|---|---|---|---|---|---|
| **Study Visit** | **14** | **15** | **16** | **17** | **18** | **19** | **20** | |
| **Weeks after IP administration** | **78** | **104** | **130** | **156** | **208** | **260** | **312** | |
| FIX Activity, LFT (CL, LL) and<br>FIX Antigen (CL)* | Starting at Week 53, monitor <u>once every 3 months$</u> | | | | Starting at Week 157, monitor <u>once every 6 months#</u> | | X | • LFT: albumin, total bilirubin, direct bilirubin, indirect bilirubin (if available), ALP, AST, ALT, total protein, GGT, LDH.<br>• FIX:C, FIX Ag.<br>• **FIX Activity should be collected in the absence of residual infused FIX**<br>• Sponsor recommends collecting specimens on Mondays through Thursdays so that the results are available prior to the weekend.<br>• *** FIX Activity and LFT testing will be conducted at local AND central lab for all time points and as clinically necessary (including samples drawn at the clinic and any mobile phlebotomy/ home health service visits)**. FIX Antigen testing will be conducted ONLY at central lab for all time points and as clinically necessary. Clinical decisions may be based on local and/or central lab results.<br>• $ Samples should be taken at weeks: 65, 78 (Visit 14), 91, 104 (Visit 15), 117, 130 (Visit 16), 143, 156 (Visit 17).<br>• # Samples should be taken at weeks: 182, 208 (Visit 18), 234, 260 (Visit 19) and 286.<br>• Utilize mobile phlebotomy/ home health services as needed. |
| Immunology (PBMC) (CL) | Only if corticosteroid treatment is needed for a suspected T-cell response. Utilize mobile phlebotomy/ home health services, as needed and if possible. | | | | | | X | • Specimens for PBMC by ELISPOT are to be collected at End of Study and if the corticosteroid treatment is needed for a suspected T-cell response.<br>• Specimens are to be collected prior to initiating the corticosteroid treatment. If not collected prior to initiation, specimens must be collected within 24 hours of administering corticosteroids.<br>• It is highly recommended that participants who are initiated on corticosteroid treatment be treated with a gastric acid reducer, preferably a proton pump inhibitor (PPI) (eg, omeprazole), or alternatively a histamine type 2 (H2) antagonist (eg, ranitidine) for the duration of the corticosteroid course.<br>• Specimens are to be collected approximately 3 weeks after steroid administration. Additional specimens may be necessary if ELISPOT has not normalized at 3 weeks and/or based on medical judgement. |
| Spare Plasma | X | X | X | X | X | X | X | Approximately 5 mL of blood will be drawn to provide approximately 2 mL of plasma, each time blood is collected for central laboratory samples. The plasma will be stored for repeat or additional testing. These samples will be destroyed at the end of the study. |
| Liver Ultrasound | | X | | X | X | X | X | All participants will undergo liver ultrasound. Imaging will be performed locally (Section 8.2.5). |
| Global Hemostasis Markers (CL) | | X | | X | X | X | X | aPTT (in seconds), INR, TAT, TGA, and D-dimer.<br>**These specimens should be collected in the absence of residual infused FIX**.<br>The tests may be conducted as clinically indicated throughout the study. |
| HJHS | | X | | X | X | X | X | To be completed by the investigator or the designee (Section 8.1.2.1). |

| **Procedures Year 2 to Year 6** | **Week 53-156 (±2 weeks)** | | | | **Week 157-260 (±3 weeks)** | | **EOS Week 312 (±3 weeks)** | • Unscheduled visits may be necessary during the study. Tests being performed are for safety monitoring purposes, for repeat safety assessments, or to repeat any blood sampling if required.<br>• During Long-Term Monitoring Period (Weeks 53 to 312), mobile phlebotomy/ home health services may be utilized as needed, when site visits are not scheduled.<br>• Early discontinuation visit should follow the procedures for Week 312 visit. The management of the participant and the risks inherent with discontinuation from the study should be discussed with the sponsor's medical monitor. |
|---|---|---|---|---|---|---|---|---|
| **Study Visit** | **14** | **15** | **16** | **17** | **18** | **19** | **20** | |
| **Weeks after IP administration** | **78** | **104** | **130** | **156** | **208** | **260** | **312** | |
| Patient-Reported Outcomes (PROs) | | X | | X | X | X | X | Participants will complete 5 instruments at the study site using an electronic tablet: Haem-A-QoL, HAL (v2), PGIC-H, HLIQ, and EQ-5D-5L. The questionnaires should be administered before dosing, treatment, or conversation between health care team and participants about their health condition. See Section 8.1.3 for additional considerations.<br>Every effort should be made to conduct the PROs according to the protocol specified timepoints. If the post-infusion PROs can't be performed at a particular visit, they should be performed at the next visit. |
| Phone call | | | | | Ongoing | | | After Week 156, phone calls every 6 months (ie, weeks: 182, 234, 286) in addition to the annual visits. Inquire about AEs and concomitant treatments (including surgeries and procedures). |
| Vector Shedding Analysis by PCR (CL) | Weekly, as needed#<br>Utilize mobile phlebotomy/ home health services as needed | | | | | | | • # Plasma, PBMC, saliva, semen, and urine: Specimens will be collected at baseline and every week after IP administration until 3 consecutive specimen test negative for the given specimen type.<br>• Semen samples can be collected at home the night before a clinic or mobile phlebotomy/ home health service visit and stored in the participant's freezer until the clinic or mobile phlebotomy/ home health service visit. |
| Any Additional Safety Tests | Ongoing | | | | | | | Additional laboratory testing may be conducted as deemed clinically necessary by the investigator to ensure safety of participants. |
| Adverse Events | Ongoing (including unscheduled visits) | | | | | | | During the Long-Term Monitoring Period (Week 53 post-infusion to EOS) the following adverse events will be collected:<br>• SAEs (including medically important events, Appendix 3).<br>• Non-serious AEs determined to be related to IP by the investigator or where causality is unknown. |
| Concomitant Medications, Surgeries, and Procedures | Ongoing (including unscheduled visits) | | | | | | | During the Long-Term Monitoring Period (Week 53 post-infusion to EOS) the concomitant therapy associated with:<br>SAEs (including medically important events, Appendix 3)<br>Non-serious AEs determined to be related to IP by the investigator or where causality is unknown.<br>All procedures and surgeries (including elective surgeries) during the Long-Term Monitoring Period are to be recorded. |
| Review of the eDiary | Ongoing (including unscheduled visits) | | | | | | | Review the eDiary; ensure appropriate entries, during unscheduled and scheduled visits. |
| Return eDiary | | | | | | | X | The eDiary will be returned during the Week 312 visit. |

Study Site: Site that will screen and follow participant post-infusion of PF-06838435 for the duration of the study.
AAV: adeno-associated virus; ADA: Anti-drug antibody; AE: adverse event; ALP: alkaline phosphatase; ALT: alanine transaminase; APRI: AST to Platelet Ratio Index; aPTT: activated partial thromboplastin time, AST: aspartate transaminase; BUN: blood urea nitrogen; CBC: complete blood count; CL: central laboratory; CPT: cell preparation tube; Cr: creatinine; DNA: deoxyribonucleic acid; ELISPOT: Enzyme-Linked ImmunoSpot assay; EOS: End of Study; EQ-5D-5L: EuroQol, 5 dimensions, 3 levels; FIX: coagulation factor IX; FIX Ag: factor IX antigen; HBcAb: Hepatitis B core antibody; GGT: gamma-glutamyl transferase; Haem-A-QoL: Haemophilia Quality of Life Questionnaire for Adults; HAL: Haemophilia Activities List; HBsAg: hepatitis B surface antigen; HBV: hepatitis B virus; HCV: hepatitis C virus; HIV: human immunodeficiency virus; HJHS: hemophilia joint health score; HLIQ: Hemophilia Life Impacts Questionnaire; ICF: Informed Consent Form; INR: international normalized ratio; IP: investigational product; IU: international

| **Procedures Year 2 to Year 6** | **Week 53-156 (±2 weeks)** | | | | **Week 157-260 (±3 weeks)** | | **EOS Week 312 (±3 weeks)** | • Unscheduled visits may be necessary during the study. Tests being performed are for safety monitoring purposes, for repeat safety assessments, or to repeat any blood sampling if required.<br>• During Long-Term Monitoring Period (Weeks 53 to 312), mobile phlebotomy/ home health services may be utilized as needed, when site visits are not scheduled.<br>• Early discontinuation visit should follow the procedures for Week 312 visit. The management of the participant and the risks inherent with discontinuation from the study should be discussed with the sponsor's medical monitor. |
|---|---|---|---|---|---|---|---|---|
| **Study Visit** | **14** | **15** | **16** | **17** | **18** | **19** | **20** | |
| **Weeks after IP administration** | **78** | **104** | **130** | **156** | **208** | **260** | **312** | |

units; LDH: lactic acid dehydrogenase; LFT: liver function tests; LL: local laboratory; mins: minutes; MRI: magnetic resonance imaging; nAb: neutralizing antibodies; PBMC: peripheral blood mononuclear cells; PGIC-H: Patient Global Impression of Change- Hemophilia; PROs: patient-reported outcomes; RNA: ribonucleic acid; SAE: serious adverse event; TAT: thrombin-antithrombin level; TGA: Thrombin generation assay; vg: vector genome; WBC: white blood cell.